

1 SAN BERNARDINO, CALIFORNIA; OCTOBER 20, 2022

2 DEPARTMENT J-6

HON. STEVEN A. MAPES, JUDGE

3 P.M. SESSION

4 APPEARANCES:

5 The Mother with her counsel,
6 RACHEL RAYMOND & KATHY WOLF, Attorneys at Law;
7 The Father with his counsel,
8 DENNIS MOORE, Attorney at Law;
9 MICHAEL KOENIG, Attorney at Law,
10 representing the Minors;
11 HANNA TAVILL, Deputy County Counsel,
12 representing the Children and
13 Family Services.

14 (STORMY L. HINKLE, Official Reporter, CSR 12343.)

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17 THE COURT: On the record on Castaneda.

18 You all state your appearances. We will start with
19 the interpreters. I have two. I will have both of you
20 identify yourselves for the record.

21 THE INTERPRETER: Maricela Micotto. ID number
22 300787. Verified as required by statute. My oath is on
23 file.

24 THE COURT: Can you identify -- she will be
25 starting, but you guys are going to rotate every 30 minutes.

26 THE INTERPRETER: Francisco Salinas, certified
27 Spanish interpreter. ID Number 331496. Verified as
28 required by statute. My oath is on file.

1 THE COURT: Go ahead.

2 MS. WOLF: Kathy Wolf, cocounsel with Rachel
3 Raymond who is not at this time at counsel table but will
4 be.

5 MR. MOORE: Dennis Moore for the father. He is
6 present in court being assisted by the Spanish interpreter.

7 MR. KOENIG: Michael Koenig, CAG, on behalf of the
8 minors who are not present.

9 MS. TAVILL: Deputy County Counsel Tavill on behalf
10 of the Department. Social worker Monger is present via
11 video.

12 THE COURT: It is my understanding that Ms. Raymond
13 is in a hearing in J5, but you would be able to go forward
14 with the first witness the County has, and you indicated you
15 were good to go and start trial at this time?

16 MS. WOLF: That's correct. I believe it was
17 Dr. Aziz, who was going to be the first witness.

18 THE COURT: That's the representation.

19 So, Ms. Tavill, go ahead and present the evidence.

20 MS. TAVILL: As a preliminary matter, I would be
21 asking the Court to amend Athena's petition, the 300(j)
22 allegation, Count 10. I would ask that instead of the
23 petition reflecting "Mother" that it reflect, "Nick
24 Castaneda, the father."

25 THE COURT: By interlineation, is there an
26 objection?

27 So you're replacing the name or adding the name?
28 Are you replacing the name or adding the name?

1 MS. TAVILL: Replacing it. Asking to strike
2 "Mother" and add the father.

3 THE COURT: Any objection to that?

4 MR. KOENIG: No.

5 MR. MOORE: No.

6 THE COURT: Waive further arraignment, reading,
7 advisal, and waive defects?

8 MR. KOENIG: Yes, sir.

9 THE COURT: Okay. That's by interlineation, the
10 Court will make that change on that allegation.

11 Rosa, can you please indicate that in the minutes,
12 please.

13 Anything else?

14 MS. TAVILL: At this time, I would ask the Court to
15 admit the following reports and all attachments into
16 evidence: The detention report, dated October 20th, 2021;
17 the jurisdiction and disposition report, dated November
18 10th, 2021; the addendum reports, dated January 6 and June
19 23rd, 2022; the 6.7 reports, dated June 23rd, August 8th,
20 August 31st, September 12th, September 19th, and
21 September 28th, 2022; and the declaration of due diligence
22 for Father Aparicio, dated November 10th, 2021.

23 THE COURT: And any objection to any of that being
24 admitted?

25 MR. KOENIG: No.

26 MR. MOORE: No.

27 MS. WOLF: No, your Honor.

28 THE COURT: Admitted -- all those items requested.

1 Anything else?

2 MS. TAVILL: I would ask the Court to permit me the
3 ability to get Dr. Aziz, and I would be calling her as my
4 first witness.

5 THE COURT: That's fine.

6 THE BAILIFF: Face the clerk, and raise your right
7 hand to be sworn.

8 THE JUDICIAL ASSISTANT: Do you solemnly state the
9 evidence you shall give in this matter shall be the truth,
10 the whole truth, and nothing but the truth, so help you God?

11 THE WITNESS: I do.

12 THE BAILIFF: Have a seat, please.

13 THE COURT: Off the record for this.

14 (Whereupon there was a pause in the proceedings.)

15 THE BAILIFF: If you can state your first and last
16 name, spelling both for the record.

17 THE WITNESS: Komal, K-o-m-a-l; last name, Aziz,
18 A-z-i-z.

19 THE COURT: You may inquire.

20 KOMAL AZIZ,
21 called as a witness by the AGENCY, was sworn and testified
22 as follows:

23 DIRECT EXAMINATION

24 BY MS. TAVILL:

25 Q. Dr. Aziz, good afternoon.

26 Where are you currently employed?

27 A. At Loma Linda University by the GMS office.

28 Q. And how long have you been employed there?

1 A. Since July 1st, 2021.

2 Q. And what is it that you do there?

3 A. I'm training to be a child abuse pediatrician, and
4 completing my 3-year fellowship. I'm a year-two fellow at
5 the moment.

6 Q. Since you're a year-two fellow, does that mean you
7 completed medical school?

8 A. Yes.

9 Q. And did -- what type of doctor are you?

10 A. So, I'm a Board-certified general pediatrician, and
11 I am licensed to practice in the State of California, and
12 since July of 2021, I am a fellow in child abuse pediatrics.

13 Q. Given you're a fellow, what does that program
14 entail?

15 A. So, it's a three-year subspecialty training in
16 child abuse pediatrics where I medically evaluate children
17 with any form of suspected maltreatment or children with
18 other diagnoses under the supervision of child abuse
19 pediatricians to gain knowledge about expertise in the
20 diagnoses of child maltreatment and accidental trauma and
21 mimickers of child abuse.

22 Q. Do you get to see patients on your own?

23 A. It's under supervision.

24 Q. And how many patients would you estimate you've
25 seen since you started your fellowship?

26 A. About 250.

27 Q. Prior to starting your fellowship, did you work
28 with any other patients?

1 A. Yes. So, I did my pediatric residency in St. Louis
2 University. That's a 3-year program, and I would guess that
3 I saw about 1,500 or more cases.

4 Q. And what education did you obtain in order to get
5 the position you are in now?

6 A. So, after high school and college, I completed five
7 years of medical school, three years of pediatric residency,
8 board certification in general pediatrics, and a California
9 State Medical License.

10 Q. Do you do any continuing education in regards to
11 pediatric child abuse?

12 A. Yes. So the fellowship is continuing medical
13 education. We have weekly longitudinal diagnostics where we
14 learn from child abuse pediatricians in the country. As
15 well as I attend courses and conferences to stay up-to-date
16 on the literature. Prior to the fellowship, I participated
17 in electives in other parts of the U.S.

18 THE COURT: For the record, Ms. Raymond is present
19 at the hearing.

20 Q. (BY MS. TAVILL:) Do you hold any board positions
21 relevant to child abuse or pediatrics?

22 A. So, I'm a certified general pediatrician, but I
23 also serve on other committees.

24 Q. What committees do you serve on?

25 A. So, outside of the multidisciplinary team meetings
26 at Loma Linda Children's Hospital, as well as the Children's
27 Assessment Center in San Bernardino County, the child
28 detriment meetings for San Bernardino County, The Child

1 Protection Team meetings for Loma Linda University
2 Children's Hospital.

3 Q. And are you being compensated for your testimony
4 today?

5 A. No. I'm on salary for my fellowship.

6 Q. Are you familiar with Elleanor, Athena, and Austin
7 Aparicio?

8 MS. WOLF: Objection as to whether or not she is an
9 expert in this field.

10 May I inquire further?

11 THE COURT: Were you gone establishing her
12 expertise?

13 MS. TAVILL: At this time -- that's fine, your
14 Honor.

15 THE COURT: Voir dire.

16 VOIR DIRE EXAMINATION

17

18 DIRECT EXAMINATION

19 BY MS. WOLF:

20 Q. So when you first saw the Castaneda child, Athena,
21 basically, you had only been in the fellowship program for
22 two months?

23 THE COURT: We can't hear you because of the
24 microphone.

25 Go ahead.

26 Q. (BY MS. WOLF:) Did you see the Castaneda children
27 in this case?

28 A. Yes, two of them.

1 Q. Would it be that you were only a fellow for two
2 months prior to seeing these children?

3 A. Roughly. Since July 1st.

4 Q. Okay. Your board certification -- is it general?

5 A. It's general pediatrics.

6 Q. And when did you take that test?

7 A. General pediatrics?

8 Q. For your board certification?

9 A. Last year, 2021.

10 Q. And are you a fellow or diplomat of the pediatrics
11 -- of the Academy of Pediatrics?

12 A. Yes.

13 Q. And what is your position in the Academy of
14 Pediatrics?

15 A. I'm a fellow.

16 Q. Is that because you are in training?

17 A. Yes, because I'm a fellow, yes.

18 Q. So what happens after you -- if you would complete
19 your fellowship in three years, what would happen after
20 that?

21 A. I would be taking my boards in child abuse
22 pediatrics, and I would be a Board-certified pediatrician,
23 as well as a Board-certified child abuse pediatrician.

24 Q. So, I don't understand.

25 So, you got a board -- you're Board-certified in
26 pediatrics, but I think you just said that you would become
27 a Board-certified pediatrician after your fellowship?

28 A. No. The board certification in child abuse

1 pediatrics after I complete my fellowship training. I'm
2 already a Board-certified pediatrician.

3 Q. And have you made any publications?

4 A. Yes.

5 Q. How many have you done?

6 A. So, I've worked on multiple research projects, but
7 at this time, two have been published.

8 Q. What are the titles of those?

9 MS. TAVILL: Objection. Relevance.

10 THE COURT: Overruled.

11 THE WITNESS: So, the first study was nocturnal
12 enureses and characteristics associated with that in
13 children in Pakistan. Nocturnal enuresis is bedwetting.
14 And the second was about autistic children.

15 Q. So none of the publications have to do with child
16 abuse; correct?

17 A. No.

18 VS. WOLF: I'm going to object to labelling her as
19 an expert in child abuse. She just -- she doesn't qualify
20 under 730 -- Evidence Code 720.

21 THE COURT: All right. Does counsel wish to -- any
22 other counsel wish to examine related to the voir dire as to
23 this witness's proposed expertise?

24 Mr. Moore?

25 MR. MOORE: Yes.

26 / / /

27 / / /

28 VOIR DIRE EXAMINATION

CROSS-EXAMINATION

BY MR. MOORE:

Q. For this year-plus that you've been at Loma Linda, you've seen 250 patients?

A. Yes.

Q. And each patient that you assessed you've been under the supervision of another doctor?

A. Yes.

Q. And how long will you be under the supervision of another doctor there?

A. So, until the completion of my fellowship.

Q. So for another . . .

A. One and a half years.

Q. One and a half years or July of 2024?

A. Basically, end of June of -- yeah, 2024, 4.

Q. Four. Okay.

MR. MOORE: No other questions.

THE COURT: Mr. Koenig.

VOIR DIRE EXAMINATION

CROSS-EXAMINATION

BY MR. KOENIG:

Q. You stated it was 250 patients you've seen at Loma Linda; is that correct?

A. And Children's Assessment Center of San Bernardino.

Q. So 250 total?

A. Yes.

1 Q. And before that, you mentioned a number 1,500. Do
2 any of them involve suspected child abuse?

3 A. Yes.

4 Q. How many?

5 A. It's a guesstimate, but I would say about 50.

6 Q. About 50 of those.

7 And during those 50, did you also receive special
8 training and education in order to handle those cases?

9 A. Yes.

10 Q. Out of the 250 patients seen at Loma Linda, how
11 many of those cases involved suspected child abuse?

12 A. So 250 are the suspected-child-abuse cases.

13 MR. KOENIG: Nothing further. Thank you.

14 THE WITNESS: Did you mean, like -- I wrote in my
15 report that they are suspected for child abuse.

16 Q. (BY MR. KOENIG:) Well, 250 that involve some
17 aspect of suspected child abuse?

18 A. Yes.

19 Q. So, 250 would be the number?

20 A. Yes, if I'm understanding your question correctly.

21 MR. KOENIG: I think you are.

22 THE COURT: Ms. Tavill.

23 MS. TAVILL: I would ask to qualify her as an
24 expert in child abuse. There is no requirement that she be
25 completed with fellowship, as outlined in Evidence Code
26 Section 7720. Dr. Aziz has shown that she is competent in
27 child abuse. She has received training, and she is well
28 versed and capable of making opinions regarding child abuse

1 on her own.

2 THE COURT: Ms. Wolf, do you have anything related
3 to 720.

4 MS. WOLF: I'm sorry?

5 THE COURT: Do you have anything on that argument
6 related to 720?

7 MS. WOLF: Yes, your Honor.

8 I don't believe she has shown that she is
9 knowledgeable. She just said she saw these cases. I don't
10 know if she knows what the outcome is with suspected abuse.
11 I'm going to read the qualifications under 730 -- 20.

12 "Persons qualified to testify as an expert if he or
13 she has special knowledge, skills, experience, training, or
14 education sufficient to qualify him as an expert on the
15 subject to which testimony relates."

16 And how can it be sufficient if she is still in
17 training, and when she first saw the children, she had only
18 been in the fellowship program for 2 months.

19 THE COURT: I understand what you're saying.
20 Basically, your argument is she hasn't completed the
21 training; however, the Court views -- and I think the view
22 of the qualification for expert is a lower standard than
23 that when you review the verbatim language. And you read it
24 into the record. Does the expert have special knowledge?
25 Yes. Special skill? Yes. Experience? Yes. Training?
26 Yes. And education sufficient to qualify -- it says him or
27 her, but it's her in this case, as an expert on the subject
28 to which the testimony relates, and the answer is "yes."

1 So the Court's going to overrule your objection,
2 but it's noted for the record. Thank you.

3 You may continue with your examination.

4 MS. TAVILL: Thank you, your Honor.

5 DIRECT EXAMINATION (resumed)

6 BY MS. TAVILL:

7 Q. Dr. Aziz, I had asked you if you had examined
8 Elleonore and Athena, and you indicated "yes?"

9 How did you become aware of Elleonore and Athena?

10 A. Athena was admitted on October 15th, 2021. The
11 primary team reached out to forensic pediatrics, meaning me,
12 but we admitted the child.

13 Q. And did you provide a consult in this case?

14 A. Yes.

15 Q. Did you provide an opinion as to the injuries for
16 Elleonore and Athena?

17 A. Yes.

18 Q. What did you use in order to form the opinion for
19 Elleonore and Athena's injuries?

20 A. I interviewed the parents at the bedside. I spoke
21 to the admitting team for both children. I examined the
22 children myself. I had communicated with other
23 subspecialties that were consulted in the case. We ordered
24 testing, including labs and imaging for the children that I
25 reviewed myself and pediatric radiology. I reviewed medical
26 records and also taking information and history from CPS and
27 law enforcement.

28 Q. Now, you indicated that you examined Elleonore and

1 Athena yourself. How many times did you examine them?

2 A. I examined Athena twice when she was hospitalized,
3 and one additional time for her hospital follow-up at the
4 Children's Assessment Center, and I examined Elleonore
5 twice, once in the hospital and once for follow-up for
6 hospital.

7 Q. And you indicated that you interviewed the parents.
8 How many times did you speak with them?

9 A. Once.

10 Q. Did you review anything after forming your opinion
11 about their injuries?

12 A. I reviewed the medical records provided by CFS, CFS
13 reports, as well as the forensic interview reports provided
14 that was available for the older brother. I also reviewed
15 the imaging provided by CFS for Elleonore.

16 Q. Does your opinion remain as outlined in the
17 pediatric note that is attached to the addendum report for
18 January 6th, 2022?

19 A. Yes.

20 Q. What opinion did you ultimately come to regarding
21 Athena?

22 A. Abusive head trauma and physical abuse.

23 Q. What about Elleonore?

24 A. Physical abuse.

25 Q. Now, I would like to talk about Athena's injuries.
26 How old was she when she first presented at the
27 hospital with injuries?

28 A. Twenty days old.

1 Q. And she presented on October 15th?

2 A. Yes.

3 Q. Prior to your hospital admission on October 15th,
4 was Athena seen by her primary-care physician?

5 A. Yes. Mom reported that primary -- primary-care
6 physician had seen her on the 13th.

7 Q. Are you aware of whether or not there were any
8 concerns about Athena at that appointment on October 13th?

9 A. Mom reported the concerns that she brought the baby
10 in to the primary-care provider was left side head swelling,
11 as well as redness inside her left eye and some bruising
12 around the eye.

13 Q. You stated left red eyeness (as spoken), I believe.
14 Did you observe Athena? And what did that mean?

15 A. So, when I clarified with Mom, I came to learn she
16 meant the blood inside the white part of Athena's eye with
17 some bruising to the eyelid around the left eye.

18 Q. Now, at that October 13th appointment, are you
19 aware of whether the mother was instructed to bring Athena
20 to the emergency department immediately?

21 A. Mother reported to me the primary-care provider
22 told her to take the child to the emergency room.

23 Q. Ultimately, the mother did do so on October 15th.
24 Does that delay cause you any concern?

25 A. Yes.

26 MR. MOORE: Objection. Relevance.

27 THE COURT: Overruled.

28 THE WITNESS: Yes.

1 Q. (BY MS. TAVILL:) Why?

2 A. Because the concerns were head swelling and redness
3 inside the eye -- blood inside the white of the eye and
4 bruising around the eyelid, and all three of these in a baby
5 that's less than one month old that's not moving around are
6 sentinel findings, sentinel injuries. So they need
7 evaluation, and I was also concerned because there were some
8 inconsistencies in the history that I was given, so those
9 were the two concerns I had.

10 Q. What were the inconsistencies that you noticed?

11 A. Mom reported that the reason for delayed coming
12 into the ED was transportation issues, and later when I was
13 asking questions, I found out the transportation issues were
14 because Dad worked on Tuesdays and Wednesdays, and so that
15 was the reason Mom didn't have access to transportation.
16 But that doesn't really explain why they could not bring the
17 child into the ED on the 14th when the father would not have
18 been working.

19 Q. So, as far as you know, there was not a valid
20 explanation for why they did not bring Athena to the
21 emergency room on the 14th?

22 A. Not that I'm aware of.

23 Q. Now, what happened when Athena arrived to
24 Loma Linda?

25 A. She was evaluated in the emergency room. So
26 Parents would have been interviewed by the emergency room
27 providers, and the child examined. She underwent testing,
28 including labs of blood and urine, and some imaging. And

1 once she was found to have head injuries, she was admitted
2 to the pediatric intensive care unit for further management
3 and medication.

4 Q. So, as far as medical professionals go, who all
5 would have examined Athena?

6 A. Given the nature of her injuries --

7 MS. WOLF: Objection. Calls for speculation.

8 THE COURT: Overruled.

9 THE WITNESS: Given the nature of her injuries, she
10 was seen by neurosurgery, trauma surgery, in addition to
11 having been evaluated by the ED providers and the pediatric
12 ICU team once admitted.

13 Q. (MS. TAVILL:) Is it typical for a child with
14 Athena's injuries to be evaluated by all of those medical
15 professionals?

16 A. Yes.

17 Q. And do you consult with those medical professionals
18 when forming your opinion?

19 A. Yes.

20 Q. Why do you do that?

21 A. Because I have training, residency training, in
22 pediatrics. I do not have specialty training in radiology
23 or neurosurgery or trauma surgery. So I rely on other
24 subspecialties to get insight into their expertise and get
25 details and find their points and clarify before I can give
26 an opinion on injuries and take a look at what could have
27 caused the injuries.

28 Q. And is this standard practice?

1 A. Yes.

2 Q. As a medical professional, were Athena's injury
3 significant, in your opinion?

4 A. Yes.

5 Q. Why?

6 A. Athena had multiple traumatic findings. These
7 involved multiple organ systems, like eye, head, bones, her
8 skin. Really, there was no clear explanation of what could
9 have caused these injuries. Her head injury, also,
10 basically, consisted of complex fractures, so there were --
11 on both sides of her head. They were wider than I would
12 expect with a short fall --

13 THE COURT REPORTER: Wider?

14 THE WITNESS: Wider. Separated.

15 So all of these are more commonly seen in cases of
16 abusive trauma than accidental, so these were significant,
17 and these were concerning to me.

18 Q. (BY MS. TAVILL:) Now, I'm going to turn your
19 attention to the handout labeled "Athena Castaneda," and I'm
20 going to ask you to flip to --

21 MS. WOLF: Your Honor, I don't believe we have a
22 copy.

23 MS. TAVILL: You do. I emailed it to you this
24 morning.

25 MS. RAYMOND: Yeah, but we provided you with paper
26 copies, so we would like a paper copy.

27 THE COURT: Are you going to address me like I
28 asked?

1 MS. RAYMOND: Oh, I am sorry.

2 MS. WOLF: We don't have a copy, and this morning I
3 was driving to court, so I didn't have a chance to --

4 THE COURT: You're getting a paper copy right now.

5 MS. WOLF: Thank you, your Honor.

6 THE COURT: Anyway, go ahead.

7 MS. TAVILL: Thank you. I would just note for the
8 Court and counsel, the first three pictures, which would be
9 on page 2 and 3 of the document are already in evidence, so
10 I will not be going over those. I did provide a copy to the
11 Court as well.

12 THE COURT: I have a courtesy copy.

13 Q. (BY MS. TAVILL:) Dr. Aziz, on -- on page 4 -- do
14 you have a copy of this with you?

15 A. Yes.

16 Q. On page 4 the picture on the left-hand side, do you
17 recognize that?

18 MS. WOLF: Your Honor, may I -- I don't have
19 numbers on the page.

20 THE COURT: They're not stamped? You're going to
21 have to show Counsel what you're referring to so they --

22 MS. TAVILL: Yes.

23 MR. MOORE: If I may inquire? Is it Athena or
24 Eleonore here?

25 MS. TAVILL: Athena.

26 THE COURT: The request is related to Athena; is
27 that correct?

28 MS. TAVILL: That's correct.

1 MS. WOLF: And what --

2 THE COURT: It's on page 4 of the packet labeled
3 "Athena."

4 MS. TAVILL: Yes.

5 THE COURT: So I have the face page. It says
6 "Athena." That's page 1. The second page starts with the
7 paragraph, "red," with one picture. The second page has two
8 photos -- I'm sorry that's the third page. I see what you
9 did. Okay. So, it's the third page of photos, but the 4th
10 page in the packet?

11 MS. TAVILL: Correct.

12 THE COURT: Is everybody on the same place?

13 MS. WOLF: Yes.

14 THE COURT: All right. Go ahead.

15 DIRECT EXAMINATION (Resumed)

16 BY MS. TAVILL:

17 Q. Dr. Aziz, do you recognize this picture on the
18 left?

19 A. Yes.

20 Q. How do you recognize it?

21 A. I believe I took it.

22 Q. And what does this picture show?

23 A. So, if you look closely, around the left eye on the
24 upper eyelid of Athena, there are bruises, both on the inner
25 corner and outer corner, and there is some bruising to the
26 lower eyelid also. Inside the eye, in the white part, there
27 is rim of redness that you can see that's around the black
28 -- black part of the eye, which is blood. So they're two

1 different injuries that we don't expect to see with no
2 explanation in a baby that's not moving around.

3 Q. Is this picture a true and accurate depiction of
4 the picture you took of Athena?

5 A. I believe so, given the printing. It's showing
6 both of the injuries.

7 MS. TAVILL: Your Honor, I would ask to admit this
8 picture of Exhibit 1.

9 THE COURT: So -- okay. This -- what I'm trying to
10 get at, there's a packet. How many exhibits do you have for
11 this packet?

12 MS. TAVILL: This packet in particular? Nine.

13 THE COURT: Is it your intent to admit each one?

14 MS. TAVILL: I would be admitting pages --

15 THE COURT: I'm only asking because the number --
16 yeah. I'm just trying to help Rosa out, so that's why I'm
17 asking.

18 MS. TAVILL: It will be pages 4 through page 8, and
19 it will be 9 exhibits.

20 THE COURT: Okay. I'm just wondering if there is a
21 way to make, like, exhibit -- like, if it could be labeled
22 as one exhibit, and then have -- since there's multiple
23 pictures on each page, write in like -- write in on the
24 actual piece of evidence like, "A" over one picture and "B"
25 on another and on another page "C" and "D." I'm just trying
26 -- would that be helpful to you, Rosa?

27 So, I don't know. That way -- so what I'm thinking
28 here is that the first couple pages you're not having as

1 evidence?

2 MS. TAVILL: No.

3 THE COURT: What I have is a courtesy for me;
4 right?

5 MS. TAVILL: Well . . .

6 THE COURT: Is this the Court's copy for an
7 exhibit?

8 MS. TAVILL: I stapled them because I thought all
9 parties were going to agree to them coming into evidence,
10 but that is not the case. I did not know how much of this
11 would be entered as an exhibit. I'm happy to provide the JA
12 with an updated copy once that is decided.

13 THE COURT: Mainly my discussion is logistical;
14 just how we're doing this and trying to make it easy. So,
15 we don't have individual pictures at this time, and counsel
16 for the mother and father -- you're not in agreement with
17 marking the whole packet because the portions of this packet
18 that say "Athena" you're objecting to; is that correct?

19 MS. WOLF: Yes.

20 MR. MOORE: Yes.

21 THE COURT: So you want it pieced apart, which is
22 fine. I want to be clear. Are you objecting to, I guess,
23 from this page forward, which is the one with the binky that
24 I talked about? There's two pictures on that page. It's
25 just one of those pictures that you're asking about?

26 MS. TAVILL: Both.

27 THE COURT: On the next page, are you asking to
28 admit that one?

1 MS. TAVILL: Yes.

2 THE COURT: Are you objecting to that one?

3 MR. MOORE: The next page would be -- is that where
4 it is labeled, "CT Head"?

5 THE COURT: The "CT head." Of course, that's one
6 picture.

7 MR. MOORE: At the moment I'm objecting, but I'll
8 see what foundation is laid.

9 So I guess we will have to peel it apart. That's
10 fine. I appreciate it. So you have not presented this
11 separately, and this is a courtesy copy or intended to be
12 marked?

13 MS. TAVILL: However the Court wishes to go about
14 it. It would be easier if the Court could mark it, and we
15 can do it that way.

16 THE COURT: Okay. So I'm -- I'm going to pull
17 apart the packet, I guess, so Rosa can mark it.

18 Any objection to that?

19 MS. WOLF: No, your Honor, but I would like to
20 inquire about foundation and authenticity --

21 THE COURT: Do what?

22 MS. WOLF: I would like to authenticate this
23 picture.

24 THE COURT: It's already authenticated. She was
25 there. She saw it. It's the same.

26 MS. WOLF: I just want to be sure when it was
27 copied that it was in the same --

28 THE COURT: It's an accurate copy?

1 MS. WOLF: Yes.

2 THE COURT: I see what you mean. She wasn't asking
3 to admit it now. I think she was asking to have it marked,
4 which is appropriate at this time.

5 MS. WOLF: Well, I would have no objections if I
6 can just have somebody to testify, yes, this is an adequate
7 depiction of what was taken, and the picture's not
8 graphically changed and whatever people do these days.

9 THE COURT: Well, it's being marked. It's not
10 being admitted. The difference is that your objection is
11 preserved because I haven't admitted it. At the time they
12 seek to admit it, you know, all that stuff.

13 MS. WOLF: Are we doing it as Exhibit 4 or Exhibit
14 Number 1?

15 THE COURT: I think you said Number 1?

16 MS. TAVILL: I don't believe there are other
17 exhibits at this point, so, yes.

18 THE COURT: Your intention is both photos?

19 MS. TAVILL: That's fine, or we can do each picture
20 an exhibit. We can do each picture as an exhibit.

21 THE COURT: We will do one "A" and one "B." We
22 will have a handwriting. I will direct the clerk to write
23 "A" over the picture that is on the left-hand side of the
24 picture, as you look at it. The picture without the binky,
25 it will be "A." The one with the binky that is longer and
26 taller vertically -- I will order that "B."

27 Any objection to that?

28 MS. WOLF: No, your Honor.

1 THE COURT: There you go. That will be Exhibit 1
2 marked for reference.

3 Your objection is preserved --

4 MS. WOLF: Thank you, your Honor.

5 THE COURT -- for all the reasons you stated. Just
6 simply marking it.

7 What else?

8 MS. TAVILL: Should I --

9 THE COURT: Wherever you're at.

10 Q. (BY MS. TAVILL:) Dr. Aziz, the left picture on
11 page 4 of this packet, was this altered any way from your
12 understanding?

13 THE COURT: You refer to that as 1A.

14 THE WITNESS: No, it's not. It is also -- this
15 picture is part of my consult load.

16 MS. TAVILL: I would ask to admit Exhibit 1A into
17 evidence.

18 THE COURT: You don't wait until the end? That's
19 fine. You can do it.

20 Now, do you have an objection? She is asking to
21 admit it.

22 MS. WOLF: No, your Honor.

23 THE COURT: Do you have an objection?

24 MR. KOENIG: No, your Honor.

25 THE COURT: So, 1A is admitted into evidence.

26 Q. (BY MS. TAVILL:) Dr. Aziz, page 4 in that same
27 packet, which is marked as Exhibit 1B, do you recognize this
28 photo?

1 A. Yes.

2 Q. How do you recognize it?

3 A. Again, I took it.

4 Q. And what is it a picture of?

5 A. So, if you look closely, the inner corner, the
6 inside of the white of Athena's left eye, on the center
7 towards the nose, also have a rim of redness that is blood
8 around the eye, so it shows the extent of bleeding around
9 the left eye.

10 Q. And does this picture appear altered in any way?

11 A. No.

12 Q. Does this picture appear to be a true and accurate
13 picture -- copy of the picture you took of Athena?

14 A. Yes.

15 MS. TAVILL: I would ask to admit Exhibit 1B into
16 evidence,

17 MS. WOLF: No objection.

18 MR. MOORE: No objection.

19 MR. KORNIG: No objection.

20 THE COURT: Admitted, 1B.

21 Q. (BY MS. TAVILL:) Now, Dr. Aziz, the next page,
22 which would be page 5. This be Exhibit 2.

23 THE COURT: Okay. That's the one that said "CT
24 head." It's a single picture?

25 MS. TAVILL: That's correct.

26 Q. (BY MS. TAVILL:) Dr. Aziz, do you have a copy of
27 this?

28 A. Yes.

1 Q. Do you recognize this picture?

2 A. Yes.

3 Q. How do you recognize it?

4 A. This is a snapshot of the CT head reconstruction in
5 3D format showing the fractures in Athena's skull.

6 MR. MOORE: Objection. Lack of foundation.

7 THE COURT: Sustained.

8 MS. TAVILL: I'm not sure what grounds it lacks
9 foundation.

10 MR. MOORE: If I may speak?

11 How does she know that is what it depicts? Did she
12 personally witness it? Did she see it?

13 THE WITNESS: Yes.

14 THE COURT: Okay.

15 MS. WOLF: Also, it's not dated. Who did this?
16 Where was it done? It definitely lacks foundation as I see
17 it.

18 THE COURT: Overruled on that ground. Okay. I
19 thought your objection was based on something else,
20 Mr. Moore, so I'll reverse my ruling. Thank you.

21 Q. (BY MS. TAVILL:) Now, Dr. Aziz, does this appear
22 to be a true and accurate depiction of the CT reconstruction
23 of Athena's skull?

24 A. Yes.

25 Q. Does it appear manipulated and altered in any way?

26 A. No.

27 Q. This is an accurate depiction, in your medical
28 opinion, regarding the condition of Athena's skull when she

1 presented to the hospital?

2 A. Yes.

3 MS. TAVILL: Ask to admit Exhibit 2 into evidence.

4 MS. WOLF: I would object on the grounds that
5 Dr. Aziz is not a radiologist, so her interpretation is not
6 as a radiologist.

7 MR. MOORE: I'm going to renew my objection for
8 lack of foundation. How does she know this is a
9 reconstruction of the X-ray?

10 THE WITNESS: Off the CT.

11 THE COURT: No, No.

12 Mr. Koenig, do you have anything?

13 MR. KOENIG: I would point out this witness has
14 already been qualified as a child-abuse forensic expert,
15 medical expert. It's my understanding from the testimony
16 that the witness has clarified that this image is what she
17 is -- one of the aspects of what she's relied upon in making
18 her expert opinion, and that she has personal knowledge of
19 what this image is, where it came from, and that it's
20 relevant to her ultimate diagnosis, so I think it is
21 relevant. I believe there is sufficient foundation to allow
22 its entry into these proceedings.

23 THE COURT: Well, okay. Is it relevant? Did she
24 rely on it? Yes. Is the foundation evidentiary? I don't
25 know -- I agree with Counsel. I'm not sure that she has
26 established how she knows this photograph -- this is clearly
27 computer-generated. I don't know how that works. So, she
28 is relying on something, which is fine, but to say that this

1 is actual evidence, the foundation objection is well taken.

2 So you can -- I'll sustain it for now, but if you
3 want to attempt to establish that.

4 Q. (BY MS. TAVILL:) Dr. Aziz, regarding Exhibit 2,
5 the picture of Athena's skull -- did you receive this from
6 someone in the medical field?

7 A. So, this CT scan -- the images are available on the
8 software, and radiology and other medical professionals,
9 doctors, can access those images and look at them
10 themselves. These are generated -- these come with 3D
11 reconstruction as part of the sequences of 3D images, so
12 yes.

13 Q. Did you consult with radiology to confirm that this
14 is the accurate depiction of Athena's skull?

15 MR. MOORE: Objection. Hearsay. Move to strike.

16 THE COURT: Overruled.

17 THE WITNESS: Yes.

18 MS. WOLF: Your Honor --

19 MS. TAVILL: I would ask to admit the picture into
20 evidence at this time.

21 MS. WOLF: Your Honor, I would object. I think the
22 radiologist -- or a report from the radiologist would be
23 accurate and would have more of a foundation than her just
24 sending this picture of this head, and she also stated on
25 the record earlier that she consulted with radiologists
26 because she doesn't have that knowledge, and that is why
27 there are radiologists because they know what they're doing
28 and how to interpret it.

1 MR. MOORE: I object as to hearsay because sounds
2 to me like she is stating it is a business record, but she
3 is not the custodian for the essential business record.

4 THE COURT: Mr. Koenig.

5 MR. KOENIG: This witness has testified, and, I
6 believe, laid sufficient foundation about what this image
7 is, where it came from, how it came into her direct
8 observation, and how she relied upon it to further her
9 expert opinion regarding whether or not this is suspected
10 child abuse or not. I believe this witness has laid
11 sufficient testimony or sufficient foundation to allow this
12 document to be admitted into evidence and for the Court to
13 rely on it.

14 THE COURT: Okay. This is what the Court's piecing
15 together from her testimony. First of all, she did consult
16 as far as her practice with radiologists to confirm that
17 this picture was Athena's skull and an accurate depiction
18 thereof, and then she testified that these images are -- I
19 am not using her word -- routine images that come generated
20 from computer software that they rely upon as medical
21 professionals that they can access. So I think with the --
22 with the overall -- I understand the business-record issue
23 you're talking about, Mr. Moore, but what I'm seeing here
24 is, in the course of practice, this doctor in her specialty,
25 accesses something that is available through the computer as
26 part of her practice that she has relied upon in any
27 ordinary means. She verified that, yeah, that was actually
28 the patient, and that's who she is looking at. It seems to

1 me that the foundation of the radiologist confirmed to her
2 that that is the depiction of the head.

3 I understand the hearsay objections that you're
4 making related to that; however, because this is something
5 that a medical professional would routinely rely on as far
6 as their practice, she is not merely regurgitating something
7 somebody told her. She is using it to form an opinion.
8 That's where the relevance part comes in and foundation part
9 comes in. Not because it is relevant, but there is a
10 foundation because the -- it sounds to me as though the
11 normal way medicine is practiced, and she took steps to
12 verify that this is accurate. The Court will overrule the
13 foundation objection.

14 MS. TAVILL: And the Court admitted it into
15 evidence?

16 THE COURT: Yes.

17 MS. TAVILL: Thank you.

18 Q. (BY MS. TAVILL:) I would like to --

19 MS. WOLF: Can I clarify that it's Number 2?

20 MS. TAVILL: That's correct.

21 Q. (BY MS. TAVILL:) I would like to turn your
22 attention to page 6 of the packet labeled "CT head," and it
23 has two pictures on the page. For court purposes, the left
24 picture will be labeled Exhibit 3A and the right picture
25 will be 3B. In regards to the left picture, which is 3A, do
26 you recognize this picture?

27 A. Yes.

28 Q. How so?

1 A. From review of imaging for Athena's head injury.
2 It is her CT scan.

3 MR. MOORE: Your Honor, I would like to have the
4 same objection as last time for the record.

5 MS. WOLF: Me, too, your Honor.

6 Also, your Honor, I am going to make an evidence
7 objection also that there is -- there's a report from a
8 person, the scan --

9 THE COURT: I can barely see you.
10 What kind of objection are you making?

11 MS. WOLF: I'm going to also object to the Best
12 Evidence Rule. The best evidence would be the actual
13 picture with the doctor's radiologist report on it.

14 MS. RAYMOND: I don't know what she looked at. The
15 best evidence is the DVD scan.

16 MS. WOLF: The best evidence would be the original
17 --

18 THE COURT: That's not required. You can use
19 secondary evidence as the best evidence. Unless I'm
20 misunderstanding the law, what is it about this that makes
21 you think it's not the best evidence, even though it's a
22 copy?

23 MS. WOLF: Because I believe there is a DVD or some
24 type of -- the actual picture. It's produced on a DVD or
25 however they produce it. This is just a reconstruction or
26 -- I don't think it's the original.

27 THE COURT: So you want to see the raw data, the
28 reconstruction used to put it in picture form?

1 MS. WOLF: Correct; your Honor.

2 THE COURT: All right. Do you have feedback or
3 response to that?

4 MS. TAVILL: Your Honor, this is essentially the
5 original. This was constructed via her scans. The scan of
6 her brain is not the best evidence. This is just as good,
7 if not better because it gives you the dimensional aspect of
8 her brain. I would also note, Best Evidence Rule applies to
9 writings typically. That's not this case.

10 THE COURT: No, but, honestly, I think photos are
11 used when doing analysis. But in any event, a copy of the
12 original is best evidence as long as there is no
13 alterations. And from what I'm seeing here, this is very
14 consistent with what the radiologist would look at. They
15 would rely upon a computer-generated image just like this as
16 opposed to a raw image.

17 I would understand if you were saying, look, the
18 radiologist relied on something else, and now it is enhanced
19 to something different, and you want to see the original.
20 But this is something all medical professionals rely upon,
21 based on the testimony, and it wasn't something that was
22 made afterward after somebody relied on something and gave
23 an opinion and then enhanced it. In which case then, I
24 would say, yeah, let's see the original. So I'm going to
25 overrule the objection. Okay.

26 Q. (BY MS. TAVILL:) Dr. Aziz, the picture labeled 3A,
27 you indicated this is a scan of Athena's skull. Does this
28 picture depict how Athena's skull looked when she came into

1 the hospital on October 15th?

2 A. At the time when the picture was taken.

3 Q. And this picture is an accurate version of what she
4 saw when she came into the emergency department, and it does
5 not appear altered?

6 A. Yes.

7 MS. TAVILL: I would ask to Admit 3A into evidence.

8 THE COURT: Ms. Wolf,

9 MS. WOLF: I want to preserve my objection.

10 THE COURT: Okay. So I want to be clear. You're
11 still objecting on the same basis?

12 MS. WOLF: Yes.

13 THE COURT: You're objecting on the same basis?

14 MR. MOORE: Yes.

15 THE COURT: Anything else?

16 MR. KOENIG: No objection.

17 THE COURT: Admitted, over counsel's objection.

18 Q. (BY MS. TAVILL:) Dr. Aziz, I would like to turn
19 your attention to page 6 of the packet. And, now, the
20 picture on the right-hand side, which has been labeled "3B."
21 Do you recognize this?

22 A. Yes.

23 Q. How do you recognize it?

24 A. Again, this is a view of Athena's skull that the CT
25 scan was done on the 3D reconstruction sequence.

26 THE COURT: Same objection?

27 MS. WOLF: Yes.

28 THE COURT: Same objection?

1 MR. MOORE: Yes.

2 THE COURT: Same ruling.

3 Q. (BY MS. TAVILL:) And does this appear to be an
4 accurate depiction of Athena's skull when you presented at
5 the emergency department?

6 A. Yes.

7 Q. Does this picture appear to be altered in any way?

8 A. No.

9 MS. TAVILL: I would ask to admit 3B into evidence.

10 THE COURT: Same objection?

11 MS. WOLF: Yes.

12 THE COURT: Same objection?

13 MR. MOORE: Yes.

14 THE COURT: Same ruling. Overruled. Admitted.

15 Q. (BY MS. TAVILL:) Now, I would like to turn your
16 attention to page 7. The page is labeled "skeletal survey,"
17 and for court purposes, they will be "4A" and "4B." On
18 "4A," on the left-hand side, and "4B" on the right-hand
19 side.

20 The picture on the left, "4A," do you recognize it?

21 A. Yes.

22 Q. How?

23 A. This is a view of Athena's first skeletal survey of
24 her legs.

25 Q. And did you review this skeletal survey when
26 consulting on Athena's injuries?

27 A. Yes.

28 Q. And did you review this skeletal survey with any

1 other doctors?

2 A. Radiology.

3 Q. And did radiology confirm your findings regarding
4 her injuries?

5 A. So the --

6 MR. MOORE: Objection. Hearsay.

7 MS. WOLF: Hearsay.

8 THE COURT: Overruled.

9 THE WITNESS: So, the arrows that you see in these
10 photos are actually drawn by radiology and are part of the
11 images that are available for us to view, so, yes, they did
12 agree with the findings that you're seeing.

13 Q. (BY MS. TAVILL:) Does the image look altered in
14 any way?

15 A. The arrows were added by radiology to help us look
16 at the findings that they were seeing.

17 Q. Do -- does this appear to be a true and accurate
18 depiction of how Athena appeared on October 15th?

19 A. Yes.

20 MS. TAVILL: I would ask to admit "4A" into
21 evidence.

22 MR. MOORE: Same objection.

23 MS. WOLF: Same objection.

24 THE COURT: Overruled.

25 MS. TAVILL: And it's admitted, your Honor?

26 THE COURT: Yes.

27 Q. (BY MS. TAVILL:) Now, the picture on the
28 right-hand side labeled "4B." Do you recognize it?

1 A. Yes.

2 Q. How?

3 A. This is one of the views from Athena's first
4 skeletal survey.

5 Q. And how did you access this scan?

6 A. We have access to radiology images.

7 Q. And did you review this image with radiology?

8 A. Yes.

9 Q. Has the image been altered in any way?

10 A. The arrows were added by the pediatric
11 radiologists.

12 Q. Is that common practice?

13 A. Yes.

14 Q. Why?

15 A. So other nonradiologist medical professionals can
16 see any subtle findings they are seeing, and they also can
17 note all the injuries that radiology is noticing.

18 Q. And does this picture reflect how Athena came into
19 the emergency department on October 15th?

20 A. Yes.

21 Q. And it does not appear altered in any way, other
22 than arrows?

23 A. No.

24 MS. TAVILL: I would ask "4B" be admitted into
25 evidence.

26 THE COURT: Same objection?

27 MR. MOORE: Same objection.

28 MS. WOLF: Same objection.

1 THE COURT: Overruled. Admitted.

2 Q. (BY MS. TAVILL:) We will move on to page 3 of the
3 packet. The page is labeled "right knee." There are two
4 pictures on the page, and I will be labelling the left
5 picture "5A" and the right picture "5B."

6 In regards to the left picture labeled "5A," do you
7 recognize it?

8 A. Yes.

9 Q. How?

10 A. This picture is also from Athena's first skeletal
11 survey.

12 Q. And how did you access this picture?

13 A. Through our electronic-medical-record software.

14 Q. And did you consult with anyone regarding this
15 image?

16 A. Pediatric radiology.

17 Q. Does the image appear altered in any way?

18 A. The arrows were added by pediatric radiologists.

19 Q. And other than that, does the image appear modified
20 in any way?

21 A. No.

22 Q. This picture is a true and accurate depiction of
23 how Athena presented to the emergency department?

24 A. Yes.

25 MS. TAVILL: I would ask to admit "5A."

26 THE COURT: Objecting?

27 MS. WOLF: Same objection.

28 MR. MOORE: Same.

1 MR. KOENIG: No objection.

2 THE COURT: Overruled. Admitted.

3 Q. (BY MS. TAVILL:) Now, turning your attention to
4 the same page but "5B," which is the right-hand-side
5 picture. Do you recognize this?

6 A. Yes.

7 Q. How?

8 A. This is, again, part of Athena's first bone survey.

9 Q. And how did you access this picture?

10 A. Through electronic-medical-record software.

11 Q. And did you consult with anyone regarding this
12 image?

13 A. Pediatric radiology.

14 Q. And does it appear to be an accurate depiction, or
15 does it appear modified in any way?

16 A. Pediatric radiology did add the arrows, but
17 otherwise, it is not altered.

18 Q. Does this image reflect how Athena presented to the
19 emergency department?

20 A. Yes.

21 MS. TAVILL: I would ask to admit "5B" into
22 evidence.

23 THE COURT: Objection?

24 MS. WOLF: Same objection.

25 MR. MOORE: Same objection.

26 MR. KOENIG: None.

27 THE COURT: Overruled. Admitted. Thank you.

28 MS. TAVILL: Now, just for Court and counsel's

1 consideration, I am going to be referencing the physician
2 note in the January 6th addendum report. It's page 13 of
3 the physician report. It's in the attachments.

4 MS. WOLF: Your Honor, may we have a hard copy?

5 THE COURT: You don't have copies?

6 MS. RAYMOND: Your Honor, I was not provided all
7 the reports or attachments --

8 THE COURT: You've said that so many times.

9 MS. RAYMOND: I know, but nobody seems to be doing
10 anything about it. That's the problem. So Mr. Moore did
11 provide me all the reports, including attachments.

12 THE COURT: So you're saying you don't have the
13 attachments?

14 MS. RAYMOND: I have it now.

15 THE COURT: From January 6?

16 MS. RAYMOND: I can pull that out right now. The
17 Health and Education Passport that was attached to the JD?

18 THE COURT: Well, it's more than that because there
19 would be physician notes. There's a whole attachment too.

20 MS. RAYMOND: Yes.

21 THE COURT: So you do have it?

22 MS. RAYMOND: Yes. Can you give me a moment? I
23 was provided it, not by County Counsel but by Mr. Moore,
24 even though I asked repeatedly.

25 THE COURT: Let me know when you have it available.
26 (Whereupon there was a pause in the proceedings.)

27 MS. WOLF: May I inquire? Is it from the passport?

28 MS. TAVILL: I'm not sure what you're referring to.

1 MS. WOLF: May I inquire what page you're referring
2 to?

3 MS. TAVILL: It's page 13 of the physician's note.

4 MS. WOLF: Would it be at the bottom of the
5 history? Is that what -- I have page 13 and 14.

6 Is it page 13 and 14?

7 MS. TAVILL: I'm not sure what she is looking at,
8 so I can't say for certain.

9 THE COURT: Well, it's not Bates-stamped.

10 MS. TAVILL: It's attached to the January 6th
11 addendum, and the top of the document indicates "October
12 18th," and the bottom indicates it was printed November 4th.

13 MS. WOLF: Okay. I do have that.

14 And, your Honor, in our exhibit book, there's a
15 courtesy copy, which is Number 14.

16 THE COURT: So you do have it?

17 MS. WOLF: It's Bates-stamped in Volume 3 that you
18 have. I'm trying to streamline it so everybody has it.

19 MS. RAYMOND: But it wasn't timely provided. That
20 would be the issue, your Honor.

21 THE COURT: All right. Go ahead.

22 MS. TAVILL: Okay. Thank you.

23 Q. (BY MS. TAVILL:) Now, I would like to take the --
24 I would like to discuss the first injury.

25 The first finding you have regarding Athena's
26 injury. It indicates she had a depressed right parietal
27 bone fracture with overlaying scalp contusion and
28 nondisplaced left parietal fracture extending from left

1 coronal suture to the lambdoid suture." What does that mean
2 in layman's terms?

3 A. So, I would point to page 6 of the photographs that
4 you were just looking at.

5 Q. So that would be Exhibits "3A" and "3B"?

6 A. Yes. So looking at "3A," this is Athena's skull,
7 if you were viewing it from the side. So this piece is the
8 front of the head, the forehead, and this piece is the back
9 of the head.

10 MR. KOENIG: Your Honor, for the record, the
11 witness is pointing to "3A" on the left side of the paper.
12 The witness pointed to left-most sphere of the photo showing
13 a forehead. And just for the record, that's just above the
14 eye socket, I believe, on that photograph. And then the
15 witness indicated to the right side of "3A," there's a
16 subpartition, again, in the little hemisphere that the
17 witness pointed to on the document.

18 THE COURT: Okay. You accurately described what
19 the Court saw.

20 THE WITNESS: So a baby this age has spaces between
21 all skull bones. In order for the brain to grow, these
22 bones are not fused. So there are spaces between bones that
23 are called sutures, and those are normal. However, this
24 line right here is --

25 MR. KOENIG: Your Honor, the witness is pointing to
26 the top most horizontal line in the middle of the skull.

27 THE COURT: So reflect.

28 THE WITNESS: -- not a normal suture. This is the

1 left parietal skull fracture. And you can see it starts at
2 one suture line and ends at the other suture line. So that
3 is what it means when they say, extending from the left
4 coronal suture to the left lamboid suture.

5 MR. KOENIG: For the record, the witness indicated
6 that this line that goes up and then slightly down that
7 connects both the two skull lines is the fracture. She
8 outlined this on the image.

9 THE COURT: So reflected.

10 THE WITNESS: The rest of the spaces that we are
11 seeing in this photo are normal sutures.

12 Q. (BY MS. TAVILL:) Now, in regards -- that was the
13 left side of Athena's head. What about the right side?

14 A. So, "3B" represents skull if you were viewing it
15 from the side and looking at the right side of the head.
16 So, on the left side of that photo is the back of the head,
17 and the right side is the forehead. The fracture is from
18 the right side of the picture that goes up. So this line
19 should not be there, and it starts at one suture line and
20 ends at the other suture line, which goes across on top of
21 the head. So this fracture, again, starts at one suture
22 line and goes all the way over to the other suture line and
23 is depressed. Depressed means the bone is curved inwards,
24 so that is marker of the force that is required to cause
25 that.

26 THE COURT: For the record, the witness has
27 identified the line that is a 45-degree curve on the lower
28 end to the right upper end toward the left and starts about

1 -- I don't know -- halfway to the top of the suture line to
2 the front going up to the midline suture to the top of the
3 skull. Is that accurate?

4 THE WITNESS: Yes. And I can label the sutures
5 too. It starts at the left coronal -- sorry. Right coronal
6 suture, it goes up to the left suture on top.

7 MS. WOLF: May you turn it up?

8 THE WITNESS: So coronal suture is where it starts,
9 and it ends at this suture.

10 MR. KOENIG: Again, the witness is indicating on
11 the right-hand side of -- right-hand side of "3B" going up
12 to the top of what would be the skull towards the top middle
13 of "3B."

14 THE COURT: That's what I saw.

15 Q. (BY MS. TAVILL:) Now, you indicated the right
16 parietal bone was dented. Is that commonplace, or does that
17 take some force to cause?

18 A. It would take force to cause.

19 Q. Is it concerning that Athena, at 21 days old, had
20 this type of injury?

21 A. Yes.

22 Q. Now, you mentioned the fractures go from one suture
23 line to the other suture line. Is that concerning?

24 A. Yes.

25 Q. Why?

26 A. There are certain characteristics of skull
27 fractures that make it more concerning for inflicted injury
28 versus accidental or injury from a short fall, for instance,

1 and these characteristics include a suture that is widened
2 or separated, which both of these sutures are. A suture
3 that's crossing -- a fracture that is crossing suture lines
4 -- these don't necessarily cross suture lines, but they do
5 extend from one suture line and end at the other. Skull
6 fractures that are multiple, and skull fractures if they're
7 bilateral, meaning both on the left and the right side of
8 the head. So there Athena, she had at least three of these
9 characteristics, so these are worrisome.

10 Q. And, given the fracture went from one suture line
11 to the other, is there a correlation between more force
12 needing to be used to cause that severe of a fracture?

13 A. Yes. The higher the force, the longer the
14 fracture.

15 Q. Is it easy to cause these types of fractures in an
16 infant? Can it be something accidental?

17 A. These fractures are not consistent with accidental
18 or short-fall injury. With short-fall injuries, the most
19 common injury is no injury to the skull. If there is a
20 fracture in the skull, it's most likely going to be single
21 parietal bone, and it would look minimal.

22 So, for example, I can point to a suture just so
23 everyone understands the types of fractures we usually see.
24 So if you look at Exhibit 2, you would see on the right side
25 that there is a tiny line next to the suture. It's not a
26 fracture, but these are the types of fractures we commonly
27 see with short fall injuries.

28 MR. KOENIG: And, for the record, the witness is

1 showing Exhibit 2. She is pointing to the left side of what
2 would be the top of the skull. Towards the top, there is a
3 comparatively small indentation towards the upper half that
4 runs about halfway across the round part of the skull. It
5 looks like an indentation, not a complete broken bone;
6 correct?

7 THE WITNESS: Right. So it's --

8 THE COURT: The Court will make the record reflect
9 that's what I saw.

10 THE WITNESS: So, typically, we see a smaller crack
11 in the skull, but we don't typically see bone pieces
12 fragmented and separated.

13 Q. (BY MS. TAVILL:) So this intense of fractures is
14 typically seen in severe car accidents or severe abuse;
15 right?

16 THE COURT: Wait. Which type of fracture? The one
17 she just talked about, or the one Athena had?

18 MS. TAVILL: The fractures Athena had.

19 THE WITNESS: The two parietal fractures that she
20 had.

21 THE COURT: Thank you.

22 Q. (BY MS. TAVILL:) Now, you touched on this a little
23 bit. Do you believe there were multiple impacts to Athena's
24 skull to cause these fractures?

25 A. I believe it is likely that she had more than one
26 impact to the head.

27 Q. And why do you believe that?

28 A. The presence of scalp contusion that's listed on

1 the right on top of the right parietal-bone fracture
2 indicates that there was an impact or trauma to that side
3 that caused injury to that area of the scalp, but she also
4 has a left parietal skull fracture. And Mom reported that
5 she had seen swelling to the left side of her head on the
6 11th and 12th of October prior to bringing her in to
7 Pediatrician's office or the ED. So those are markers of
8 impact to the head. And the direction of the two fractures,
9 right and left side are different, so that also leads me to
10 believe there were two different impacts.

11 Q. What do you mean by, "They were in different
12 directional patterns"?

13 A. They are going in different directions. So, for
14 instance, across the skull and from top and then curving to
15 the side, so they're not seeming to be converging on top of
16 the head or, you know, at the back of the head, so they're
17 going into different directions.

18 Q. So would that lead you to believe that she was hit
19 in at least two spots?

20 A. That leads me to believe she had blunt-force trauma
21 to different spots of the head.

22 Q. You mentioned the overlaying scalp contusion
23 contributed to your thought that it was multiple impacts to
24 Athena's head. What exactly is overlaying scalp contusion?

25 A. That means scalp injury on top of the fracture. So
26 injury on top of the bone, not involving the bone itself,
27 so, skin and soft tissues underneath.

28 Q. And what would be the mechanism to cause a scalp

1 Contusion?

2 A. Blunt impact.

3 Q. Now, in regards to the fractures that Athena
4 sustained to her skull, can you date those in any way?

5 A. They appear to be acute. Skull fractures don't
6 show the same healing as other long bones of the body, so
7 it's hard to give a definitive timeline, but the presence of
8 scalp contusion on top of the right side fracture, and the
9 history of swelling that Mom reported on the left side
10 indicates they're acute. So they were recent injuries.

11 Q. And you -- okay.

12 So, in your opinion, her skull fractures did not
13 have any signs of healing?

14 A. No, they did not.

15 Q. And the best way to diagnose a time frame for skull
16 fractures is based on the history of injury symptoms of the
17 child; is that accurate?

18 A. Yes, and characteristics, like swelling on top, or
19 edema on top of the fracture.

20 Q. Now, I would like to -- is there anything else you
21 would like to touch on regarding Athena's skull fractures?

22 MR. MOORE: Objection. Calls for a narrative.

23 THE COURT: Experts are allowed to narrate.
24 Overruled.

25 THE WITNESS: I think we covered the fractures.

26 Q. (BY MS. TAVILL:) I'm going to move on to the
27 second injury that was listed in your findings.

28 It's. Small subdural hematomas underlying the right

1 skull fracture and along the left temporal and occipital
2 lobes. Trace hemorrhage along the posterior falx and left
3 tentorium. Small focus of extra axial hemorrhage at the
4 right frontal vertex. Suggestion of tiny bifrontal
5 microhemorrhages.

6 What does that mean?

7 A. All of these different areas of bleeding around --

8 THE COURT: Okay. Just for the record, your client
9 went to the restroom. We're going to take a break in a few
10 minutes.

11 Would it be okay if this question gets answered?

12 Thank you. Go ahead.

13 THE WITNESS: So, these are basically, a detailed
14 description of exactly the areas where bleeding around
15 Athena's brain was noticed.

16 Q. (BY MS. TAVILL:) And what would be the cause to
17 bleeding in Athena's head like this around her brain?

18 A. So, the cause is acceleration/deceleration
19 mechanism. That causes shearing of the vessels that are
20 attaching to brain and, basically, the dura. So if the
21 brain moves at a different speed with blunt trauma or
22 shaking, for instance, then the skull in the dura -- the
23 vessels will tear, and this will cause the bleeding.

24 Q. Is this bleeding in her head likely to be from
25 birth-related trauma?

26 A. No.

27 Q. Why not?

28 A. Because she has other findings that go along with

1 the bleeding that we're seeing, so blunt -- there's evidence
2 of blunt-force trauma to the head. So this is likely from
3 blunt-force trauma to the head.

4 Q. And can you determine when exactly Athena began
5 having all this bleeding in her head?

6 A. So, on imaging, CT and MRI, this appeared to be
7 acute or fresh blood. This was a recent injury and not an
8 old injury.

9 Q. And how could you tell it was acute blood versus
10 old blood?

11 A. They appear different on scans. So on CT, fresh or
12 acute blood would appear whiter or hypo -- hyperintense, but
13 then it starts to age, it will appear darker on some of the
14 sequences and on CT, then that's how we determine the old
15 versus recent or acute.

16 Q. Is there anything else you would like to say
17 regarding the blood in Athena's brain before moving on to
18 other injuries?

19 A. It's a significant amount of bleeding.

20 THE COURT: Okay. Let's be in recess.

21 Come back in about 15 minutes.

22 (Whereupon a recess was taken.)

23 THE COURT: Back on the record.

24 All counsel's present that was here earlier. Our
25 interpreter is here and Francisco.

26 And you're still under oath. Do you understand
27 that?

28 THE WITNESS: Can you say that again?

1 THE COURT: You are still under oath. Do you
2 understand that?

3 THE WITNESS: Yes.

4 THE COURT: All right. Ms. Tavill.

5 MS. TAVILL: Thank you, your Honor.

6 Q. (BY MS. TAVILL:) Now, Dr. Aziz, the third injury
7 listed was normocytic anemia due to blood loss. Is that
8 just blood loss?

9 A. Yes.

10 Q. Is that a common or expected result due to the
11 significance of Athena's injury?

12 A. It's expected with the amount of bleeding, yes.

13 Q. Now, the fourth injury that was listed is the mild
14 sulcal prominence in the left temporal lobe suggesting mild
15 atrophic changes with associated cortical hemorrhage or
16 laminar necrosis and mild edema or encephalomalacia?

17 A. Encephalomalacia (pronunciation).

18 Q. Thank you. What exactly does that mean?

19 A. So, this is a description of injury to brain tissue
20 itself and edema in the brain tissue --

21 Q. Okay --

22 A. -- involving the left temporal lobe of brain.

23 Q. The sixth injury is, MRS shows mildly reduced "N"
24 double "A" and mildly elevated C-H-O, slash, CR, in the
25 bilateral parietal white matter. What does that mean?

26 A. So, these are findings that were seen on MR
27 spectroscopy, and these indicate injury to brain cells in
28 both right and left parietal areas of Athena's brain.

1 Q. So the fourth injury and the sixth injury listed in
2 your findings are both related to injuries to Athena's brain
3 tissue; is that right?

4 A. Yes.

5 Q. And what does injury to brain tissue mean?

6 A. So, of all the findings or injuries that we see
7 with head injuries or head trauma -- this is what we worry
8 about the most. Brain tissue once injured or, you know --
9 it will not regenerate, and it does not heal. So those
10 brain cells are dead, basically. So what this means in
11 terms of Child is long-term consequences from the injury,
12 such as developmental delays, neurocognitive deficits. So,
13 this is worrisome.

14 Q. So, effectively Athena's brain tissue died and will
15 not regenerate?

16 A. In these areas, yes.

17 Q. And how many areas did she have brain tissue that
18 died?

19 A. So one -- at least the left temporal lobe and then
20 the -- both the parietal areas where the fractures were,

21 Q. And what would be the mechanism or cause brain
22 tissue to die in someone Athena's age?

23 A. So it can be the head trauma or, for instance, the
24 finding on MRS that shows the axonal injury. We also see
25 with acceleration/deceleration from shearing of these axons
26 that are brain cells. So, the same mechanism. Either
27 shaking or blunt trauma to the head.

28 Q. Can you date when exactly brain tissue dies?

1 A. No, because these changes are evident on MRIs if
2 they're done with some delay from the brain injury itself.
3 Because with time, they become more noticeable on MRI, but
4 it's hard to figure out exactly what time those injuries
5 started because they don't become apparent on MRIs right
6 away, so we rely on other factors to determine time.

7 Q. So when Athena first presented to the emergency
8 department, was this brain tissue that had died apparent?

9 A. No. MRIs were not done the same day as her arrival
10 to the ED.

11 Q. Before we move on to the fifth listed injury, is
12 there anything else you think is important regarding the
13 dead brain tissue that Athena sustained?

14 A. No, I think we have covered it.

15 Q. The fifth injury that was listed is the mild
16 suboccipital soft tissue edema seen on the MRI spine. You
17 mentioned this a little bit in the last-listed injury. So,
18 what does that mean?

19 A. This is soft tissue edema. I previously talked
20 about the edema in the brain or brain tissue. This is
21 different. The suboccipital area is the area in the back of
22 the head just above the neck, so this indicates soft-tissue
23 injury to that area.

24 Q. And so it indicates an injury to that area
25 specifically. And how do you determine -- how did you
26 determine that Athena had this injury?

27 A. This was seen on MRI spine.

28 Q. And what did it look like on the MRI to denote she

1 had this injury?

2 A. It was fluid collection seen in that area.

3 Q. Similarly with this injury, can you date it or
4 determine when specifically it occurred?

5 A. So, we know that it was there when the MRIs were
6 done. We do not know how this injury was prior to the
7 completion of the MRIs because we didn't have -- it's a
8 snapshot in a point in time when the MRI were done. So I
9 can't comment if it was worse than before or if it was
10 better than before.

11 Q. Now, is there anything else you would like to
12 discuss about that injury prior to moving on to the next
13 one?

14 A. This can be explained by the head trauma that she
15 experienced.

16 Q. Okay. And, then, the seventh injury listed is the
17 left periorbital bruising with the conjunctival hemorrhage
18 of the left eye.

19 You mentioned a bit about this, but what is it?

20 A. The subconjunctival hemorrhage of left eye is the
21 bleed that is in the tissue that's the white of the eye, and
22 this is an abnormal or concerning finding in a baby that's
23 not moving, that's not mobile, so this is what is called
24 sentinel injury. This can be caused by blunt impact or
25 trauma to the eye itself, or if the chest or the rib cage of
26 the child was squeezed, that would cause backwards flow of
27 blood to the head and neck area causing those tiny vessels
28 to pop, and that would cause a lot of bleeding. So this is

1 a concerning finding. The periorbital bruising on the left
2 side is bruising to the eyelids to the left eye, so that is
3 consistent with blunt impact to the left eye.

4 Q. I'm going to take that one bit at a time.

5 You mentioned sentinel injuries. What exactly is a
6 sentinel injury?

7 A. So, this term in literature refers to a seemingly
8 minor injury that are associated with inflicted trauma or
9 child abuse. So literature shows that many times in serious
10 cases of child abuse or fatal cases of child abuse, these
11 injuries were previously documented and were missed by
12 medical providers. In 40 percent of cases of fatal child
13 abuse, these injuries have been documented to have been
14 missed.

15 MS. WOLF: I object. Lacks foundation for the
16 literature.

17 THE COURT: Do you have a response?

18 MS. TAVILL: Your Honor, the witness is testifying
19 to her knowledge of the study. She is an expert relying on
20 medical studies.

21 THE COURT: Overruled. Thank you.

22 MS. TAVILL: Now --

23 THE COURT: You can cross-examine on that.

24 MS. WOLF: I am.

25 Q. (BY MS. TAVILL:) Now, the eye bruises and the
26 blood in Athena's eye, given those are typical of sentinel
27 injuries, would that be a red flag that she suffered from
28 physical abuse?

1 A. Both of these are sentinel injuries, and what this
2 means is, had she come in with either of these alone, that
3 means we need to evaluate her for more injuries, or she
4 needs evaluation.

5 Q. Now, do you believe the eye bruising or the blood
6 in the eye occurred during Athena's birth?

7 A. No. Mom denied the presence of either before -- I
8 believe she noticed this on October 2021, so she said it was
9 not there before, so no.

10 Q. And you can't date the eye bruising, can you?

11 A. No.

12 Q. Why not?

13 A. Because bruises are different in individuals. Each
14 child will bruise differently depending on the severity of
15 the impact, the type of tissue involved. They would also
16 break down the blood that is causing the bruising
17 differently, based on their metabolism, and, skin color, so
18 a lot of factors impact how a bruise looks on any given time
19 on a child, so we can't date them.

20 Q. And as far as you're aware, was there ever a
21 history given by the parents to explain why she had this eye
22 bruising?

23 A. No. Mom reported that she didn't know what caused
24 it on -- when she first noticed it, so she had no
25 explanation, and Dad didn't either.

26 Q. And did the eye bruise appear to heal or worsen as
27 time went on?

28 A. So, Mom reported that it had worsened from the

1 first time she noticed it. She said the first day or first
2 few days it appeared as black dots around the eyes or
3 scratches, but she didn't realize that it was bruising, and
4 she said it had worsened by the time the child was seen in
5 the ED. However, from her admission to the ED, by the time
6 I saw her, it seemed to have slightly improved, so that's
7 how it changed.

8 Q. And, then, could Athena have caused the eye
9 bruising to herself?

10 A. No.

11 Q. Could she have caused the blood in her eye to
12 herself?

13 A. No.

14 Q. Why not?

15 A. Because she is a 21-day-old baby. She does not
16 have any developmental capabilities required to generate
17 enough force. Children, when they're less than one-month
18 old, they don't have the developmental capabilities to
19 injure or scratch or rub their eyes, so it is not possible
20 at all.

21 Q. And could -- there's been some speculation that one
22 of Athena's siblings could have caused this eye bruising.
23 Is that possible?

24 A. Children do not cause the blunt-force trauma or
25 impact that would cause bruising to this protected area, no.
26 And Parents also denied reporting or noticing any such
27 behavior.

28 Q. If Mother reported that Ellenore, the toddler,

1 patted Athena, the newborn baby, would a pat be forceful
2 enough to cause the eye bruising or the blood in the eye?

3 A. No.

4 Q. And you mentioned that the bleeding in the white of
5 Athena's eye would be caused by blunt-force trauma or
6 squeezing or compression of the child. What exactly would
7 that entail?

8 A. So, blunt impact to the eye, such as punch, for
9 instance. It is a forceful impact. That can explain the
10 bruising as well as the subconjunctival hemorrhage.
11 Subconjunctival hemorrhage -- it can be caused by, you know,
12 squeezing of the rib cage or chest because that causes
13 pressure build up inside, forcing blood to not flow into the
14 head and neck area, so they can have the tiny vessels pop in
15 the whites of the eye, so that's another method they can get
16 the subconjunctival hemorrhages.

17 Q. Regarding the blood in Athena's eyes. Are you able
18 to date that injury?

19 A. It is basically --

20 MR. MCORE: Objection. Asked and answered.

21 THE COURT: I think you did ask and answer that.

22 MS. TAVILL: I asked about the bruising in the eye,
23 not the blood in the eye.

24 THE COURT: Overruled.

25 THE WITNESS: Mom said she noticed it first on the
26 12th.

27 Q. (BY MS. TAVILL:) So, given Mom first noticed the
28 bruise -- or the blood in the eye on the 12th, would you

1 deduce that the injury happened either on the 12th or before
2 then?

3 A. Yes.

4 Q. Was the amount of blood in Athena's eye concerning
5 for any reason?

6 A. It was. Typically, subconjunctival hemorrhages
7 appear as red dots in the white of the eye. She seemed to
8 have bleeding entirely -- basically, surrounding her whole
9 pupil. So the amount is more than we typically see.

10 Q. Did the presence of blood in her eye give any
11 weight to this -- her injuries being physical abuse?

12 A. It is a suspicious finding alone with no
13 explanation. It is -- that finding alone is suspicious.
14 So, in addition to other findings that she had, and the
15 periorbital bruising was suspicious for abuse.

16 Q. Before we move on to the next injury, is there
17 anything else you would like to discuss regarding the eye
18 bruise or the blood in the eye?

19 A. It could not have been caused by birth.

20 MR. MOORE: Your Honor, I missed that last answer.

21 THE COURT: Eye bruising could not have been caused
22 by birth.

23 MR. MOORE: Thank you.

24 Q. (BY MS. TAVILL:) Dr. Aziz, given the two skull
25 fractures that we discussed earlier, you indicated that
26 those would have had to be at least two impacts to cause the
27 differing fractures. Would the injury to Athena's eye
28 result from either of those impacts, or would this need to

1 be third impact?

2 A. This is an additional impact focused on the eye,
3 which is where the bruising is, and it's different from the
4 skull findings.

5 Q. Now, in regards to the eighth injury listed,
6 cortical irregularities involving the left distal tibial
7 metaphysis, the right proximal tibial metaphysis, and the
8 right distal femoral metaphysis, what does that mean?

9 A. So, I will point out to Exhibit 4A. These injuries
10 are irregularities in long bones of Athena's legs, right and
11 left. These are suspicious for fractures, and, typically,
12 they don't appear very, you know, dramatic or -- they appear
13 subtle on X-rays. It is the nature of these injuries
14 because babies bones are not all the way calcified, so some
15 part of the bone is cartilage that is going to be calcified
16 later on as they grow, which does not show on X-rays. So
17 you cannot see the entire part of the bone. These fractures
18 are shearing or tears between the noncalcified part that's
19 not apparent and the calcified part, which is the bones that
20 we can see on X-rays. And of all the injuries we see in
21 child abuse, these are the ones that are most specific of
22 inflicted trauma based on mechanism that's required to cause
23 these injuries.

24 Q. And what is the mechanism required to cause these
25 types of fractures?

26 A. So these are seen in children where a caregiver
27 would shake the child so violently, and the child's arms and
28 legs are flailing around causing enough force to shear and

1 cause the tear between the cartilage and the actual
2 calcified bone. It can also be seen if the arm or leg is
3 shaken, pulled, or twisted so forcefully so as to cause a
4 fracture.

5 MR. KOENIG: For the record, the witness was
6 holding out during the last question and the one before it,
7 what has been marked as images "4A" and "B" entitled
8 "Skeletal Survey" that shows Athena's legs.

9 THE COURT: It does reflect that.

10 Q. (BY MS. TAVILL:) Now, how -- looking at this, I
11 can't tell what the fracture is. So, how exactly could you
12 diagnose the fracture based on these injuries?

13 A. They appear usually as a little chip off the bone.
14 Because the only piece of the fracture that we can actually
15 see is a tiny piece of calcified bone that has chipped away
16 from the bone. So they appear to be those tiny tears or
17 chips. They are called buckle fractures, corner fractures,
18 CML, so there's multiple names for this type of fracture.

19 I do want to mention that in the report that you
20 referenced, it says, "Left distal tibial metaphysis. And
21 pediatric radiology added an arrow to it, but on follow-up
22 imaging, that was found not to be a fracture. On follow-up
23 images it did not show any signs of healing.

24 MS. WOLF: Objection. Lacks foundation. I don't
25 know what report you're talking about.

26 THE COURT: Overruled.

27 THE WITNESS: It is listed in all the image reports
28 in my hospital follow-up visit. We did find the fractures

1 on the distal part of the right femur. Meaning, above the
2 right knee, below the right knee, so proximal part of right
3 tibia, and distal part of right tibia, so above her right
4 ankle, so three different locations for these fractures.

5 Q. (BY MS. TAVILL:) So just to sum it up, Athena
6 had -- on her right leg, she had a fracture above her knee,
7 just below her knee, and then just above her ankle?

8 A. Yes.

9 Q. This is not a common accidental injury. This has
10 to be violent --

11 A. This does not happen with blunt impact, blunt
12 force, or falls. This has to be violent shaking, violent
13 twisting, or traction of the leg or arm, so it is very
14 specific for abuse.

15 Q. And is there an age group that this injury is
16 specific for?

17 A. Less than 6 months of age because the bones are not
18 calcified yet.

19 Q. So Athena being 21 days old, this injury -- we're
20 expected to see if she went through trauma, this type of
21 injury?

22 A. Yes.

23 Q. And, now, with the leg fractures, can you date
24 these with any specificity?

25 A. So, these fractures do not heal as other long-bone
26 fractures where the bone is broken in the white of the
27 calcified parts of the bone. So we can -- basically, the
28 best way to date them is repeating follow-up images and

1 making sure that they are showing healing and have healed at
2 some point, but we can't really say when this fracture
3 occurred.

4 Q. Okay. Did Athena's injury -- or did Athena's right
5 leg fractures eventually heal?

6 A. Yes.

7 Q. And was there any history given by the parents to
8 explain how she sustained these right leg fractures?

9 A. No.

10 Q. So, at this point, her right leg fractures would be
11 abuse -- physical abuse?

12 A. Yes.

13 Q. Now, is there anything else you would like to add
14 about that injury?

15 A. I will add the only reports of children with these
16 types of fractures that are documented in the literature are
17 babies who had very difficult breach deliveries where
18 providers had to manipulate the leg and pull or, you know,
19 grab at the leg, or if a child had club foot, which is a
20 congenital deformity, and leg or foot was being helped to
21 place in a cast. So those are the documented instances of
22 accidental causes of this injury with good history of what
23 caused this, so it's not seen in accidental trauma with no
24 explanation.

25 Q. As far as you're aware, Athena was born --

26 A. Mom denied any sort of instrumentation during
27 Athena's birth. There is no history of trauma related to
28 birth.

1 Q. And Athena didn't have a club foot?

2 A. No, she did not.

3 Q. Now, is there anything else you would like to
4 discuss regarding those injuries?

5 A. No.

6 Q. So the ninth injury listed is the multiple areas of
7 subpleural. What does that mean, and is it still a valid
8 finding?

9 A. There was some finding on -- or cortical areas that
10 seemed a little abnormal or concerning on the initial
11 skeletal survey for Athena. Which, on repeat and follow-up
12 X-rays, is not -- were proven not to be fractures, so these
13 are normal findings.

14 Q. So, at this point, that injury is no longer -- that
15 --

16 A. That was not an injury.

17 Q. Thank you.

18 Now, I am not sure if the Court wants to stop here
19 before I go into my next section?

20 THE COURT: It's probably a good segue.

21 You're going into what?

22 MS. TAVILL: I'm going into -- my next section will
23 be the parents' conversations with the doctor.

24 THE COURT: Okay. It's probably a good time to
25 stop. You will be excused until October 23th. Let's see
26 the time I had. We had October 20th for the next date at
27 8:30 and 1:30.

28 I'm going to do 8:30 and 1:30. I have other cases,

1 but this will take priority since we're in progress. And,
2 then, I'll just have the doctor check in with Ms. Tavill to
3 see what time she really has to be here, based on the
4 Court's calendar that day. It will probably be later in the
5 morning.

6 So you're excused for now; subject to recall.
7 You'll still be under oath when you come back.

8 And, Ms. Raymond, you wanted to put something on
9 the record earlier before lunchtime, but I said it was after
10 my staff's break, so if you want to put anything on the
11 record.

12 MS. RAYMOND: I am not --

13 THE COURT: You're not going to --

14 MS. RAYMOND: Oh, wait. There is the Dr. Grogan
15 thing.

16 THE COURT: Yeah, what's up with that?

17 MS. RAYMOND: What's that?

18 THE COURT: Sorry. Tell me.

19 MS. RAYMOND: I couldn't hear what you said.

20 So, I just -- at the break, I spoke to his
21 assistant about when he is available to come in person. I
22 know he is available to come in person on the 21st.

23 THE COURT: You're not here that day; is that
24 correct?

25 MS. TAVILL: No.

26 THE COURT: Does he have other dates?

27 MS. RAYMOND: October 17th.

28 THE COURT: I was told Judge Poncin has you --

1 MS. RAYMOND: This is ongoing, so it would have to
2 be ongoing.

3 THE COURT: You're committed to that. Those dates
4 were the 17th, 18th, and 19th?

5 MS. RAYMOND: Right, for the other trial.

6 THE COURT: So what other dates?

7 MS. WOLF: Your Honor, did I hear correctly that
8 Dr. Aziz is coming back on the 20th?

9 THE COURT: That's correct.

10 MS. RAYMOND: The assistant said she was going to
11 text me with a few other dates, so I can look at my phone.

12 THE COURT: Dr. Aziz is going to take the whole
13 day?

14 MS. TAVILL: Yes.

15 THE COURT: So don't schedule anybody else on the
16 20th then.

17 MS. RAYMOND: Your Honor, is it possible --

18 THE COURT: I anticipate it will take all day since
19 you're not done on direct.

20 MS. RAYMOND: Your Honor, is it at all possible
21 that Dr. Grogan can testify during the morning session?

22 THE COURT: Of what date?

23 MS. RAYMOND: The 21st?

24 THE COURT: Ms. Tavill's not here on the 21st.

25 MS. RAYMOND: Oh. Wednesday, November 2nd?

26 THE COURT: Yeah. I got something else. I'm going
27 to bump it.

28 MS. RAYMOND: We can go at 1:30?

1 THE COURT: I'll do 8:30 and 1:30 or whatever.

2 MS. TAVILL: I'm not --

3 THE COURT: If it is more convenient for 1:30.

4 MS. TAVILL: I'm not available that day. I have a
5 CACI.

6 THE COURT: Okay. Do you have another date?

7 MS. TAVILL: I'm looking.

8 MS. RAYMOND: Your Honor, she doesn't -- I don't
9 have another date.

10 THE COURT: Let's get one. If you can get other
11 dates available, you can email Rosa and CC each other. That
12 way it's not ex parte. And, then, we will see what we have
13 on calendar and schedule Dr. Grogan, and, then, while we're
14 at it, then we will do a full day for him. So we will have
15 to pick some other dates. So I want to start blocking out
16 other dates. So I think that would probably be good once we
17 nail down Dr. Grogan's date.

18 Do you have other witnesses that are going to
19 testify?

20 MS. TAVILL: Yes.

21 THE COURT: Let me go back to the calendar because
22 that means that after Dr. Aziz is done, if we have a date
23 available, we can, kind of, fill it, right?

24 What date did I say for Dr. Grogan?

25 MS. TAVILL: You didn't.

26 THE COURT: Oh, that's right. Okay. Well, are you
27 all here Thanksgiving week?

28 MS. WOLF: I'm on jury duty. I continued jury duty

1 this week to Thanksgiving.

2 MR. MOORE: I'm not available on Tuesday.

3 THE COURT: Counsel's not available on that week.
4 I could try to bump some other trials and put it on and get
5 it done.

6 How many -- besides Dr. Aziz, how many witnesses do
7 you have? The social worker --

8 MS. TAVILL: I would guess three other witnesses.

9 THE COURT: Do you anticipate all day with them?

10 MS. TAVILL: Half day probably.

11 THE COURT: I'm thinking, maybe, I can try to do
12 the 17th. I can interrupt Dr. Aziz and do the 17th.

13 MR. MOORE: Of October?

14 THE BAILIFF: Judge --

15 THE COURT: Oh, this is my afternoon. I can't do
16 that now. We were told not to.

17 The 24th I can squeeze it there or the 31st of
18 October?

19 MR. MOORE: I can't do October 24th.

20 THE COURT: How about the 31st?

21 MS. WOLF: I can't do the 31st.

22 THE COURT: How about the 1st of November?

23 MS. WOLF: The first weeks.

24 THE COURT: I'll do 8:30 and 1:30 on November 1st.
25 It will be later in the morning, like 11:00. Put it on the
26 8:30 calendar. I'm reserving that day.

27 Maybe the 10th of November.

28 MS. WOLF: I am available, your Honor.

1 MR. KOENIG: Yes.

2 THE COURT: I'll squeeze that -- I've got another
3 trial, but I'll give priority to this one, and I've got the
4 14th of November.

5 MS. WOLF: I can do that.

6 THE COURT: Yeah.

7 MR. MOORE: I can come later in the morning on the
8 14th.

9 THE COURT: That's fine. So I want to pick a bunch
10 of days because it feels like this trial will take between 8
11 and 10 days, so I want to get a bunch here so I'm not ending
12 up past the holiday season. And, then, the 15th -- I'll
13 give this priority -- is that a good day?

14 MS. WOLF: The 15th I have two hearings.

15 THE COURT: I'm sorry. Did you say "yes" or "no"?

16 MS. WOLF: I have two hearings on the 15th.

17 MR. KOENIG: I'm okay on the 15th as long as we're
18 done by 3:00.

19 THE COURT: We set a few dates -- oh, the 30th of
20 November. I've got the 28th of November.

21 MS. WOLF: I'm available.

22 THE COURT: How about the 28th and 30th of
23 November. Are those good dates? And I would entertain --
24 I'll just throw it out there and see if Dr. Grogan is
25 available on the date that proceeds the balance of CFS case.
26 I would entertain going on a day that we had available if it
27 accommodates schedules. Is there any objection to that?

28 MS. WOLF: No, your Honor. I always agree for

1 taking a witness out of order just to accommodate
2 everybody's schedule.

3 MS. TAVILL: As long as Dr. Aziz is done and can
4 watch Dr. Grogan, I'm fine with that.

5 THE COURT: So after she is done testifying and
6 . . .

7 MS. TAVILL: And that she is available to watch his
8 testimony.

9 THE COURT: That's fine.

10 All right. We will do it that way. We have a
11 bunch of dates.

12 MS. WOLF: Can we confirm those dates?

13 THE COURT: Do you think I remember any of them?

14 MR. KOENIG: I have 11-20 -- sorry. October 20th,
15 November 1st, November 10th, November 14th, November 28th,
16 November 30th. So, 10-20, 11-1, 11-10, 11-14, 11-28, 11-30.

17 MR. MOORE: Can we throw in 11-15 also?

18 THE COURT: Somebody wasn't available.

19 MS. WOLF: I have two hearings that day.

20 THE COURT: And then -- you know, I have December
21 1st but no p.m. I can do 8:30 and 1:30 on December 1st too.

22 MS. WOLF: I'm available.

23 THE COURT: We will do all of these backup dates,
24 and then December -- I can -- I don't know. Do you think
25 I'm setting too many dates? That's 6 or 7 dates. That
26 should be enough; right?

27 MR. KOENIG: Was December 1st an actual date or no?

28 THE COURT: It's an actual date. I just did

1 another date. I'm giving enough days so -- Dennis, you look
2 like you want to say something.

3 MR. MOORE: That's fine.

4 THE COURT: Is it too many days?

5 MR. MOORE: No, it's fine.

6 THE COURT: As we get closer, I can vacate. I just
7 want to do it while things are fresh in our memories.

8 Anything else?

9 MS. RAYMOND: I don't want to be the reason why I'm
10 keeping the deputy or any other court people, but I feel
11 like a record was made without a presence.

12 THE COURT: I asked if you want to make it.

13 MS. RAYMOND: I'm thinking it's not a good idea for
14 me to --

15 THE COURT: I said yes, yes.

16 MS. RAYMOND: Because --

17 THE COURT: Yes.

18 MS. RAYMOND: Because I was --

19 THE COURT: You don't have to say why. Just say
20 what you have to say.

21 MS. RAYMOND: Okay.

22 THE COURT: Just get to it.

23 MS. RAYMOND: Okay. I'll get to it.

24 The issue is when I -- I brought up the issue when
25 she called me a liar on the record. That was the only
26 thing, and I brought it up out here. That's what I brought
27 up, and I know Mr. Koenig was on the phone because he was
28 doing that, and I wasn't angry. I wasn't -- I just wanted

1 her to discuss with me to get past that.

2 THE COURT: Let me stop you real quick. I should
3 say this. I don't know if the conduct ascribed to you
4 amounts to -- don't be offended, but I am telling you --
5 amounts to a possible criminal charge. I don't know.

6 MS. RAYMOND: I can tell you --

7 THE COURT: Don't say anything. I'm just going to
8 advise you and remind you that -- you don't have to say
9 F'ing BS to me.

10 MS. RAYMOND: Not you. I am talking -- No, I
11 wouldn't -- I am not saying it to you.

12 THE COURT: You just mouthed that.

13 MS. RAYMOND: No, it's bull- --

14 THE COURT: Yeah, exactly. I saw you mouth, F'ing
15 BS.

16 MS. RAYMOND: Because it's not to you. She knows I
17 barley touched her arm --

18 THE COURT: Counsel, where are you? Where are you
19 right now?

20 MS. RAYMOND: Where am I?

21 THE COURT: Yes.

22 MS. RAYMOND: I'm in court. I get it.

23 THE COURT: Do you think that's appropriate --

24 MS. RAYMOND: Not at all.

25 THE COURT: -- to sass me as --

26 MS. RAYMOND: I was saying it to her. That's all.

27 THE BAILIFF: Take a breath. You're interrupting
28 the judge.

1 THE COURT: I'm trying to give you advice as -- not
2 as -- how do I put? Just an advisal, not advice. That if
3 you say anything, there's a record, and because of a
4 potential allegation that could be made against you and any
5 type of -- it could be the Bar, it can be criminal or civil
6 court, whatever. I'm advising you. You have no obligation
7 to respond because all I did is say, let's move forward and
8 be nice. That's all I'm ordering. So you're not compelled.
9 I'm not compelling you to explain yourself.

10 So if you want to stop where you are, that's good.
11 If you want to, you're a lawyer, if you feel like you want
12 to explain the rest of it, which you were about to get into
13 because how you may have grabbed her arm or touched her arm,
14 or whatever. You can do that, or you can let it be, and
15 let's move forward. You can talk to your partner if you
16 want to see what she advises you to do.

17 MS. RAYMOND: I'm letting -- I'll let it be. I
18 just --

19 THE COURT: But you're objecting?

20 MS. RAYMOND: Objecting to?

21 THE COURT: You're objecting to what Ms. Tavill
22 said and what the bailiff said and whatever anybody else
23 said happened?

24 MS. RAYMOND: I'm not sure what the bailiff said.

25 THE COURT: It was the bailiff who was here
26 previously.

27 MS. RAYMOND: I believe he didn't say -- I don't
28 know. I wasn't in here, if it is going to become

1 something --

2 THE COURT: I don't remember the details of what he
3 said. You can ask for a transcript. I want people to be
4 nice to each other, even though we're in tension in the
5 court.

6 MS. RAYMOND: I mean --

7 THE COURT: That's a big deal to me. I know we're
8 high tension, especially in cases like these, but there is
9 no reason that we have to deal with each other in a way that
10 really diminishes the credibility of the Court and
11 proceedings and also the civility between lawyers, and there
12 is no reason to do that. And so that is why I say, "Let's
13 move forward," and that's what we're doing. Your cocounsel
14 made a beautiful record indicating that was certainly the
15 intent, and that's how she wants things to be as well. And
16 so we're just moving forward. Okay?

17 MS. RAYMOND: I understand. I just want the Court
18 to understand.

19 THE COURT: I'm going to say this. I'm not sure
20 what to do about it, so I'm still thinking about it. I'm a
21 little bothered when you sat back and you mouthed the words,
22 F'ing BS.

23 MS. RAYMOND: F'ing bull- --

24 THE COURT: Yes. That disturbs me. I can't
25 quantify or articulate why it disturbs me, other than we're
26 in court, and it's inappropriate. I don't understand the
27 mouthed vocalization that I could see. You were not looking
28 at me. You were looking off to the side. I agree with

1 that. In any event, we're in the courtroom.

2 I'm going to tell you. Even though it's a water
3 under the bridge, the other thing I am going to tell you, it
4 makes it look like you have an inability to control your
5 emotional reaction to something, not realizing that's how it
6 looks. So you really need to examine your behavior. You
7 need to learn to apply a filter in this proceeding or these
8 proceedings in a courtroom, right, so that there can be
9 decorum and credibility in the court, that it's not a
10 free-for-all. It's not -- like, it's not, like, you know
11 some show, if you will, where everybody's taking the ride
12 for entertainment. That's not it at all. We need to really
13 maintain that type of decorum, so please work on that.
14 Okay?

15 MS. RAYMOND: I understand, and I've never had this
16 issue with County Counsel, and I've been doing this for 11
17 years. I talk to them. We may not agree on the record, but
18 we're able to talk. I even had a nice conversation with
19 Ms. Tavill on the phone. I reached out to her because I
20 wanted to have a good connection, you know.

21 THE COURT: Maybe it's --

22 MS. RAYMOND: I don't know what happened in
23 between, but when she called me a liar, that's upsetting.

24 THE COURT: Maybe just some self-reflection or
25 something. Maybe talk to your cocounsel. Maybe you're not
26 wrong at all. I don't know. I do know what you did here
27 with me --

28 MS. RAYMOND: Oh, that's wrong. There's no

1 question.

2 THE COURT: Okay. We will see you next time.
3 (Whereupon the proceedings were concluded.)
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